

NHS Devon Integrated Care Board Service Specification

Service Specification No:	11
Service:	Clozapine Agreement
Commissioner Lead:	Vicks Johnson (Director of Commissioning)
Provider Lead:	
Period:	1 st April 2026 – 31 st March 2027 (3 months' notice will be required to terminate this contract by either party)
Date of Review:	31 st March 2027, or earlier if superseded by updated guidance.

Introduction/Population Needs

This specification applies to patients registered with GP practices in Devon, and for whom Devon Partnership Trust (DPT) is the secondary care provider of mental health services. It should be noted that patients for whom Livewell Southwest is the secondary care provider of mental health services are excluded from this service.

National/local context and evidence base

This guideline does not require General Practitioners (GPs) to prescribe clozapine and relates to only provision of phlebotomy associated with clozapine and collection of pre-dispensed sealed packages of medication from GP practices.

Clozapine is an oral antipsychotic drug that can only be prescribed for treatment resistant schizophrenia. Patients, prescribers, and dispensing pharmacies must be registered with an approved monitoring service, and patients must undergo regular blood monitoring. This is because there is a significant risk of neutropenia which can progress to agranulocytosis (fatalities have occurred). The risk is greatest at the start of treatment, so blood tests are mandatory every week for the first 18 weeks, every 2 weeks between the 19th and 52nd week, then 4 weekly thereafter.

Most DPT patients are registered with the Clozaril Patient Monitoring Service (CPMS) if taking clozapine tablets or with the Denzapine Monitoring Service (DMS) if taking clozapine suspension.

Clozapine can only be prescribed by doctors and dispensed by pharmacies that are registered with CPMS or DMS. Within the Devon Joint Formulary, clozapine is listed as a "hospital only drug" and therefore should only be prescribed by psychiatrists and dispensed by local acute trust pharmacies.

NICE guidance CG178 recommends "offer clozapine to people with schizophrenia whose illness has not responded adequately to treatment, despite the sequential use of adequate doses of at least two different anti-psychotic drugs. At least one of the drugs should be a non-clozapine second – generation anti-psychotic." Adherence to this recommendation is measured as part of the national schizophrenia audit.

DPT Mental Health services are keen to make clozapine as accessible to patients as possible and, for some patients, allowing them to attend their GP practice for blood tests and/or for collecting their clozapine supplies which can have a positive effect on the patient's compliance with

treatment. There are risks to taking clozapine and it is important that GP practice staff are aware of these risks and are willing, and adequately supported, to take on this role.

In view of current work nationally, the Clozapine Service Specification acknowledges that the working together between primary and secondary care is paramount in the current health climate.

Aims and objectives of service

The aim of this service/specification is to ensure that there is a robust process for the taking of blood tests and the collection of clozapine for individuals who are prescribed clozapine and who are stable.

Population covered by this service

Residents of NHS Devon ICB who receive mental health services and specifically clozapine supply from DPT.

Included

- All individuals prescribed clozapine by specialist services who attend GP surgeries to have bloods taken and collect supplies of clozapine.

Excluded

- Individuals at practices where Livewell Southwest is the secondary care provider of mental health services, and
- Any individual who arrives in NHS Devon ICB from another geographical area. In this instance, this will require a direct referral to secondary care for assessment.

There are a group of individuals, currently known to mental health services, who will remain under the Assertive Outreach Team due to complexity. Generally, these individuals will not be attending GP surgeries for their phlebotomy services.

Specialist Responsibilities

Psychiatrist responsibilities

- Registration of self and patient with CPMS/DMS
- Arrange initial blood test and physical check (including ECG)
- Take appropriate action on red or amber blood results
- Act on information from CPMS/DMS concerning overdue blood tests
- Prescribe clozapine.
- Prescribing any dose changes and letting relevant people know
- Monitor patient's mental health
- Update patient information with CPMS/DMS and the Medicines Optimisation Team.

Care Co-ordinator responsibilities

- Monitor patient's mental health
- Arrange for phlebotomy services and supply of clozapine to the patient, either via specialist services or GP practice.
- Liaise with GP practice to ensure patient's compliance with medication and arrangements for blood tests and collecting medication
- Collect unused clozapine supplies from patient's home and arrange return to pharmacy

- Inform the Clozapine Pharmacy Technician and psychiatrist if the patient stops taking their medication
- Inform the GP Practice and Medicines Optimisation Team if there is a change of Care Co-ordinator or psychiatrist.

DPT Medicines Optimisation Team responsibilities (in collaboration with Trust Pharmacy Department)

- Check blood status on CPMS/DMS
- Inform Care Co-ordinator or GP Practice if blood test is overdue or extra tests are required
- Dispense clozapine and arranging delivery to GP Practice:
 - Medication must be sealed in an appropriate package, which clearly identifies the patient and treatment period outside which the medication may not be collected
- Liaise with GP practice and mental health staff when circumstances relating to the person change (e.g. change in blood test frequency)
- Arrange destruction of returned clozapine
- Provide advice and support to Mental Health staff and GP Practice staff as required.
- Provide GP practices with support in obtaining blood testing kits to avoid the use of local acute trust laboratories (where relevant).

GP Responsibilities

This service requires no prescribing or clozapine monitoring from the GP; the service only requires the GP practice to provide a phlebotomy service and facilitate collection from the practice of sealed medication packages.

GPs may decide to delegate some of these tasks to members of their team; however, the responsibility for ensuring the following are undertaken remains with the GP:

- Ensure a timely reply is sent to specialist in response to request for blood sampling and package distribution.
- Take blood samples in a timely manner within the designated timescale:
 - Arrange phlebotomy appointments with the patient
 - Send blood samples to CPMS/DMS using the equipment provided
 - Re-order CPMS/DMS equipment as needed
- The following arrangements to facilitate clozapine collection from the GP practice:
 - Accept delivery of the sealed medication packages containing the patient's clozapine supplies from pharmacy
 - Store the sealed medication packages in a secure place (preferably a locked cupboard) while waiting collection by the patient (or their representative)
 - **Maintain a secure collection log** (including patient/representative name, date, time and staff member releasing the pack) for each sealed clozapine package to ensure safe, auditable handover.
 - Allow collection of each sealed medication package by the patient (or their representative) during the agreed treatment period only (as stated on the pack), unless specifically agreed with specialist services
 - Alert the Care Co-ordinator if the patient (or their representative) does not collect the clozapine supply during the treatment period (as detailed on the package label)

- Return uncollected sealed medication packages to the supplying pharmacy (telephone first)
- Include clozapine on the patient medication record for information and appropriate alerts
- Inform the Care Co-ordinator if the patient transfers to another GP Practice.

Patient Responsibilities

- Tell the GP or specialist if they have any concerns or do not understand any aspect of treatment
- Attend specialist, GP and blood test appointments
- Collect their sealed medication packages from the GP surgery during the agreed treatment period
- Seek medical attention if they become acutely ill for any reason or if they experience constipation
- Talk to their doctor as soon as possible if they become pregnant or are planning a pregnancy
- Report side effects to their Care Co-ordinator or psychiatrist
- Tell their community pharmacist that they are taking clozapine when buying any over-the-counter medicine
- Store their medicines safely and securely.

Financial Considerations

Each provider (GP) contracted to provide this in-house phlebotomy service to patients will receive £5.23 per blood test, and in addition there will be a cost of £3.14 for facilitation of collection of pre-dispensed sealed medication packages by each individual. For the purposes of clarity, this equates to £365.45 per patient per year for Year One patients and £139.72 per patient per year for Year Two onwards patients.

Claims will be processed using established and existing mechanisms for ICB Commissioned Services (LES) activity capture within each ICB area. Initial (6 month) payments may need to be based on a separate one-off activity collection exercise until this is incorporated in existing processes.

DDRB will be used as the start point for annual 'inflationary' review, with any deviation from DDRB uplift requiring clear rationale.

In the event of a pandemic or other 'level 4 health' event, commissioner and LMC will meet urgently to review this specification giving consideration to:

- a. suspension of some or all elements of it,
- b. redirection of patients to alternative provider(s),
- c. in the event of actioning either a) or b) determining appropriate income protection arrangements for GP provider